



Agri-Pet Veterinary Service
1447 Millville Shandon Road
Hamilton, Ohio, 45013
1447 Millville Shandon Road

10-03-2026

Client: Lindsay Klug
Address: 1515 Brookcrest DR
Hamilton, Ohio, 45013
Patient: Sparky
Species: Canine (Dog)
Breed: Pitbull

Patient ID: 143977
Phone Number: Lindsay: 513-807-4414
Age: 9 years 2 days
Sex: FS
Color: Black

The following is a copy of the medical records for Sparky.

If you have any questions, please do not hesitate to call us at (513) 863-2306.

Tuesday the 8th of October 2024

📄 Health Status

Weight(lb): 78.00

💊 Medication

Prescribed By: Ann Everett

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

📄 History

Sparky has DJD both stifles, is managed on Gabapentin 300mg by mouth BID. She was recently seen at King's Veterinary for her annual exam and they noticed a broken tooth and recommended a dental, possible extraction. She is UTD on vaccines, on Simparica Trio monthly for HW/flea/tick prevention. She has a history of elevated liver enzymes.

📄 Plan

Recommend dental, gave estimate, may need extraction of 208

📄 Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: Overweight

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth have enamel wear, 208 has moderate tartar

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Normal ambulation with no apparent lameness.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal; bladder palpates normally.

Neurologic: No apparent abnormalities

 Assessments

DJD both stifles
Historical elevated liver enzymes
Needs a dental

Friday the 11th of October 2024

 Medication

Prescribed By: Jana Teets
EXTERNAL DISPENSE PRESCRIPTION x 60
Gabapentin 300 mg Give 1 capsule by mouth every 8 to 12 hours as need for discomfort Refill: 2 (Two)

Wednesday the 27th of November 2024

 Health Status

Weight(lb): 78.60

Thursday the 28th of November 2024

 Diagnostic Result

Requested By: David Kraushar
Supplier: IDEXX Reference US
Reference: US791-DR27628
Hematology (1) Outcome:
Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
RBC	8.87	M/ μ L	5.39	8.7	
Hematocrit	56.1	%	38.3	56.5	
Hemoglobin	20.5	g/dL	13.4	20.7	
MCV	63	fL	59	76	
MCH	23.1	pg	21.9	26.1	
MCHC	36.5	g/dL	32.6	39.2	
% Reticulocytes	1.5	%			
Reticulocytes	133	K/ μ L	10	110	

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
<i>The appropriateness of the regenerative response should be evaluated considering the degree of anemia and reticulocytosis (see guidelines below).</i> <i>Degree of bone marrow response (reticulocytes K/uL):</i> <i>Mild 110-150</i> <i>Moderate 150-300</i> <i>Marked >300</i> <i>View the VetConnect Plus Differentials for additional information.</i>					
Reticulocyte Hemoglobin	23.5	pg	24.5	31.8	
WBC	11.9	K/ μ L	4.9	17.6	
% Neutrophils	69.5	%			
% Lymphocytes	24.0	%			
% Monocytes	4.1	%			
% Eosinophils	2.3	%			
% Basophils	0.1	%			
Neutrophils	8.271	K/ μ L	2.94	12.67	
Lymphocytes	2.856	K/ μ L	1.06	4.95	
Monocytes	0.488	K/ μ L	0.13	1.15	
Eosinophils	0.274	K/ μ L	0.07	1.49	
Basophils	0.012	K/ μ L		0.1	
Platelets	380	K/ μ L	143	448	
Remarks	No abnormalities seen upon microscopic blood film review by technician.				

Diagnostic Result

Requested By: David Kraushar

Supplier: IDEXX Reference US

Reference: US791-DR27628

Chemistry (2) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Glucose	92	mg/dL	63	114	
IDEXX SDMA	Unable to determine	ug/dL		14	
Unable to determine due to gross hemolysis and /or lipemia.					
Creatinine	0.9	mg/dL	0.5	1.5	
BUN	21	mg/dL	9	31	
BUN: Creatinine Ratio	23.3				
Total Protein	7.3	g/dL	5.5	7.5	
Albumin	3.7	g/dL	2.7	3.9	
Globulin	3.6	g/dL	2.4	4	
Albumin: Globulin Ratio	1.0		0.7	1.5	
ALT	71	U/L	18	121	
ALP	1,188	U/L	5	160	
Bilirubin - Total	0.5	mg/dL		0.3	
Hemolysis Index	4+				
Index of ++++ may increase AST and CPK by 25-50%, and decrease ALP by >50%, decrease Total and Direct Bilirubin by 25-50%, and decrease SDMA by 10-25%.					
Lipemia Index	4+				
Index of 4+ may decrease ALT, AST and Direct Bilirubin values by >50%.					
SPECIMEN REQUIRED ULTRACENTRIFUGATION DUE TO GROSS LIPEMIA.					

Tuesday the 3rd of December 2024

Health Status

Weight(lb): 78.00

B.C.S: 3.0/5.0

Dental: 2/4

CRT: 1-2 sec

MM: Pink

Attitude: BAR

Physical Exam

Heart okay, Lungs okay, MM pink, BAR. Mild to moderate dental tartar. Slab fracture on upper left PM4

Assessments

Moderate dental tartar.

Elevated ALP which has been previously noted several times on prior bloodwork and ultrasound has also been performed. Sparky was on Ursadiol. ALP and ALT have improved, however there was a hemolytic index of 4+ which may decrease ALP and ALT by 50%

Plan

Due to anesthesia, your pet may act more quiet/groggy and uncoordinated than normal. To prevent injury, please monitor and confine your pet after pick up on the day of the procedure. Take care especially around stairs.

On the day of pick up, your pet may not want to eat. This is normal. If your pet seems interested, they may eat about half their normal amount of food in the evening. Do not feed your pet if they have vomited or are drooling excessively. Small amounts of water may be given throughout the afternoon and evening. Resume normal feeding the following morning.

For any tooth extractions it is important to feed your pet softened or canned food for the next 10-14 days while the incisions in their mouth heals. All sutures are absorbable.

Your pet should not be allowed to play, tug, or chew on toys/bones during this time.

Wait at least 4 weeks before starting any tooth brushing or giving dental chews as preventative care.

Therapeutic / Procedure

Nail Trim

Plan

IV catheter, IV fluids, Anesthesia, Dental scale and polish.

Therapeutic / Procedure

Canine - Dental Exam, Cleaning and Polishing

Specifics:

ASA Category? PS 2 Px w mild systemic disease (or very young or very old

Pre-anesthetic bloodwork? Yes, elevated ALP

Pre-med Acepromazine (10mg-mL) 0.06 mLs administered IM plus Atropine 1.5 mLs administered SQ

Effect of Premed? mild

IV catheter placed prior to induction? Yes Location? Right cephalic

Catheter size? 22

IV fluid type? LRS

IV fluid drip set 15 drops-mL

IV fluid Rate 1 drops per 1second(s).

Induction Agent Propofol (10mg-mL) 14 mLs total , slow IV to effect

Complications with induction Nil smooth induction no apnoea

Size E-Tube? 9

Anesthetic circuit? Rebreathing

Maintenance: Iso/O2

Procedure: Dental scale and polish

Large slab fracture of 208. Dental radiographs are normal of remaining teeth and no periodontal disease.

Extracted 208 with gingival flap technique. Closed gingiva with 4-0 monocril simple interrupted pattern

 Medication

Prescribed By: David Kraushar

Buprenorphine 0.3mg/ml Injection (per ml) x 0.8

Gave 0.8 mls IM

Monday the 30th of December 2024

 Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 60

Gabapentin 300 mg Give 1 capsule by mouth every 8 to 12 hours as need for discomfort Refill: 2 (Two)

 Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 60

Ursodiol 250 mg Give 1 tablet by mouth every 12 hours Refill: 2

Tuesday the 31st of December 2024

 Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR28063

Hematology (1) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
RBC	8.88	M/ μ L	5.39	8.7	
Hematocrit	58.3	%	38.3	56.5	
Hemoglobin	21.1	g/dL	13.4	20.7	
MCV	66	fL	59	76	
MCH	23.8	pg	21.9	26.1	
MCHC	36.2	g/dL	32.6	39.2	
% Reticulocytes	1.7	%			
Reticulocytes	151	K/ μ L	10	110	
The appropriateness of the regenerative response should be evaluated considering the degree of anemia and reticulocytosis (see guidelines below).					
Degree of bone marrow response (reticulocytes K/ μ L):					
Mild 110-150					
Moderate 150-300					
Marked >300					
View the VetConnect Plus Differentials for additional information.					
Reticulocyte Hemoglobin	24.4	pg	24.5	31.8	
WBC	13.0	K/ μ L	4.9	17.6	
% Neutrophils	63.9	%			
% Lymphocytes	19.1	%			
% Monocytes	3.2	%			
% Eosinophils	13.6	%			
% Basophils	0.2	%			
Neutrophils	8.307	K/ μ L	2.94	12.67	
Lymphocytes	2.483	K/ μ L	1.06	4.95	
Monocytes	0.416	K/ μ L	0.13	1.15	
Eosinophils	1.768	K/ μ L	0.07	1.49	
Basophils	0.026	K/ μ L		0.1	
Platelets	295	K/ μ L	143	448	

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Polychromasia	SLIGHT				
Anisocytosis	SLIGHT				
Remarks					
Slide reviewed microscopically by a hematology technician.					
No parasites seen					
No WBC morphology abnormalities seen.					

Diagnostic Result

Requested By: Ann Everett
Supplier: IDEXX Reference US
Reference: US791-DR28063
Chemistry (2) **Outcome:**
Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Glucose	72	mg/dL	63	114	
IDEXX SDMA	Unable to determine	ug/dL		14	
Unable to determine due to gross hemolysis and /or lipemia.					
Creatinine	0.8	mg/dL	0.5	1.5	
BUN	26	mg/dL	9	31	
BUN: Creatinine Ratio	32.5				
Phosphorus	6.4	mg/dL	2.5	6.1	
Calcium	9.7	mg/dL	8.4	11.8	
Sodium	145	mmol/L	142	152	
Potassium	5.1	mmol/L	4	5.4	
Na: K Ratio	28		28	37	
Chloride	106	mmol/L	108	119	
Total Protein	7.5	g/dL	5.5	7.5	
Albumin	3.3	g/dL	2.7	3.9	
Globulin	4.2	g/dL	2.4	4	
Albumin: Globulin Ratio	0.8		0.7	1.5	
ALT	96	U/L	18	121	
AST	53	U/L	16	55	
ALP	1,460	U/L	5	160	
Bilirubin - Total	0.7	mg/dL		0.3	
Bilirubin - Unconjugated	0.4	mg/dL		0.2	
Bilirubin - Conjugated	0.3	mg/dL		0.1	
Cholesterol	430	mg/dL	131	345	
Hemolysis Index	4+				
Index of ++++ may increase AST and CPK by 25-50%, and decrease ALP by >50%, decrease Total and Direct Bilirubin by 25-50%, and decrease SDMA by 10-25%.					
Lipemia Index	4+				
Index of 4+ may decrease ALT, AST and Direct Bilirubin values by >50%.					
SPECIMEN REQUIRED ULTRACENTRIFUGATION DUE TO GROSS LIPEMIA.					

Friday the 3rd of January 2025

History

Current meds:
Ursodiol 250 mg tabs- 1 tablet by mouth every 12 hours
Denamarin?

Assessments

CBC- mildly elevated HCT, mild reticulocytosis, mild eosinophilia, all else within normal limits
Profile- Elevated ALP which has been previously noted several times on prior bloodwork but is increased compared to last

bloodwork. Hyperbilirubinemia, hypercholesterolemia. Mild hyperglobulinemia. Mild hypochloremia. Mild hyperphosphatemia is likely artifact secondary to hemolysis of the sample. Hemolytic index 4+, lipemia index 4+

Plan

Recommend internal med consult and abdominal ULS. (Last ULS was performed at King's and reviewed by Sonopath). Increasing ALP and hyperbilirubinemia raise concern for worsening of gall bladder disease. Sparky is already on Ursodiol. We may also want to consider starting a low fat diet such as Science Diet I/D lowfat or RC Lowfat GI diet. Eosinophilia is present. Recommend fecal Dx + Giardia to screen for intestinal parasites. HW4dx on 9/20/24 at Kings was negative.

Tuesday the 8th of April 2025

Health Status

Weight(lb): 81.50

Medication

Prescribed By: Hannah Mabis

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Medication

Prescribed By: Hannah Mabis

EXTERNAL DISPENSE PRESCRIPTION x 60

Ursodiol 250 mg Give 1 tablet by mouth every 12 hours Refill: 10 (ten)

Medication

Prescribed By: Hannah Mabis

EXTERNAL DISPENSE PRESCRIPTION x 60

Gabapentin 300 mg Give 1 capsule by mouth every 8 to 12 hours as need for discomfort Refill: 2 (Two)

History

Sparky presented for her annual exam and update Rabies vaccine. She is doing well overall and the owner doesn't have any concerns. The owner reports gabapentin works well for her DJD.

Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: Overweight

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Normal ambulation with no apparent lameness.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal; bladder palpates normally.

Neurologic: No apparent abnormalities

Assessments

DJD both stifles

Historical elevated liver enzymes

Overweight

Plan

Rabies 3yr

HWT

Recommend HWP, flea/tick prevention

Dhp3yr and Lepto can be given by tech when they come due later this year.

Diagnostic Result

Requested By: Hannah Mabis

Supplier: IDEXX VetLab Station- IVLS

Reference: US791-DR29626

Serology (5) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Heartworm Antigen	Negative				
Ehrlichia spp.	Negative				
Lyme (Borrelia burgdorferi)	Negative				
Anaplasma spp.	Negative				

Monday the 19th of May 2025

Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Tuesday the 10th of June 2025

Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Thursday the 10th of July 2025

Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Medication

Prescribed By: Hannah Mabis

EXTERNAL DISPENSE PRESCRIPTION x 60

Gabapentin 300 mg Give 1 capsule by mouth every 8 to 12 hours as need for discomfort Refill: 2 (Two)

Thursday the 7th of August 2025

Health Status

Weight(lb): 77.90

History

The owner has been out of town for a few days and came home last night. When she came home, the owner noticed a large lump on the underside of Sparky's neck. The sound of Sparky's bark has changed as well. She is still eating and drinking normally. The owner noticed a wheezing sound when she was breathing this morning, but that seems to have resolved.

Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: 7/9

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Normal ambulation with no apparent lameness.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

6cm firm, deep mass palpated on ventral aspect of cranial cervical region to the left of midline. Acellular sample obtained on FNA attempt.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal; bladder palpates normally.

Neurologic: No apparent abnormalities

Assessments

Cervical mass

 Plan

Recommend referral. Warned owner Sparky may need a CT to visualize the mass and determine the structures involved.

Thursday the 14th of August 2025

 Medication

Prescribed By: Ann Everett

Carprofen 75mg Tablets (per tablet) x 20

GIVE 1 TABLET(S) BY MOUTH TWICE DAILY. GIVE WITH A MEAL.

 Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 26

AMOXICILLIN/CLAVULANATE 500 MG/125 MG TABLETS GIVE 1 TABLET(S) BY MOUTH EVERY 12 HOURS UNTIL GONE. GIVE WITH FOOD. REFILL: 0 (ZERO)

 Medication

Prescribed By: Rx/Technician

Amoxicillin/Clavulanate 500mg/125mg mg Tablets (per tab) x 3

Give 1 tablet(s) by mouth every 12 hours until gone. Give with food.

 Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Tuesday the 16th of September 2025

 Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Monday the 13th of October 2025

 Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Friday the 7th of November 2025

 Medication

Prescribed By: Hannah Mabis

EXTERNAL DISPENSE PRESCRIPTION x 60

Gabapentin 300 mg Give 1 capsule by mouth every 8 to 12 hours as need for discomfort Refill: 2 (Two)

Tuesday the 10th of February 2026

📍 Health Status

Weight(lb): 79.30

📖 History

O noticed pt. has been limping on front R leg for about a week now, limp seemed a little worse last week but owner is still noticing it

O has noticed when she stands her back legs are really close together, owner has noticed her slipping some and her hind end will fall

Pt. will still jump up on the couch with lots of encouragement, slower to getting up after laying down

O gives gabapentin 300mg BID and Ursodiol 250 mg BID

O would like NT today //LC

💊 Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 60

GABAPENTIN 300 MG GIVE 1 CAPSULE BY MOUTH EVERY 8 TO 12 HOURS AS NEED FOR DISCOMFORT REFILL: 2 (TWO)

💊 Medication

Prescribed By: Ann Everett

Grapiprant 60mg Tablets (per tab) x 14

GIVE 1 TABLET BY MOUTH EVERY 24 HOURS FOR ARTHRITIS PAIN

🔧 Therapeutic / Procedure

Nail Trim

📖 Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: 7/9

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Both stifles are thickened and bowed with decreased ROM. Weight bearing RF lameness as well.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal; bladder palpates normally.

Neurologic: No apparent abnormalities

Assessments

Historical elevated ALP on bloodwork, treated currently with ursodiol and clinically doing well.
DJD

Plan

Continue gabapentin
Start Gallprant treatment trial and monitor response.
Recommend obtaining new baseline bloodwork (CBC/Profile) before continuing Gallprant long term. Last bloodwork in our records is from MedVet in August 2025.

Monday the 23rd of February 2026

Medication

Prescribed By: Rx/Technician
Librela 20mg Injection (66.2-88.2 lbs) x 1
GAVE 1 ML SQ LEFT SHOULDER
Batch: 826096exp01JUN26 (Exp. 06-01-2026) x 1

Tuesday the 24th of February 2026

Diagnostic Result

Requested By: Ann Everett
Supplier: IDEXX Reference US
Reference: US791-DR34599
Hematology (1) **Outcome:**
Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
RBC	8.28	M/ μ L	5.84	8.95	
Hematocrit	56.3	%	41	60	
Hemoglobin	20.9	g/dL	14.6	21.7	
MCV	68	fL	62	76	
MCH	25.2	pg	22.1	26.7	
MCHC	37.1	g/dL	32.3	38	
RDW	16.9	%	10	19	
% Reticulocytes	1.8	%			
Reticulocytes	149	K/ μ L	21	140	
Reticulocyte Hemoglobin	26.0	pg	23.8	28.3	
WBC	11.2	K/ μ L	5.8	16.2	
% Neutrophils	71.7	%			
% Lymphocytes	22.4	%			
% Monocytes	3.3	%			
% Eosinophils	2.5	%			
% Basophils	0.1	%			
Neutrophils	8.03	K/ μ L	3.004	9.741	
Lymphocytes	2.509	K/ μ L	0.98	4.2	
Monocytes	0.37	K/ μ L	0.145	0.736	
Eosinophils	0.28	K/ μ L	0.141	1.927	
Basophils	0.011	K/ μ L		0.1	
Platelets	374	K/ μ L	120	412	
Remarks					

No abnormalities seen upon microscopic blood film review by technician.

🔍 Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR34599

Chemistry (2) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Glucose	91	mg/dL	63	114	
IDEXX SDMA	11	µg/dL		14	
<i>SDMA and creatinine are within the reference interval: impairment of GFR is unlikely. Recommended next step: evaluate complete urinalysis.</i>					
Creatinine	1.2	mg/dL	0.5	1.5	
BUN	30	mg/dL	9	31	
BUN: Creatinine Ratio	25.0				
Phosphorus	4.6	mg/dL	2.5	6.1	
Calcium	10.8	mg/dL	8.4	11.8	
Sodium	144	mmol/L	142	152	
Potassium	5.5	mmol/L	4	5.4	
Na: K Ratio	26		28	37	
Chloride	107	mmol/L	108	119	
Total Protein	6.9	g/dL	5.5	7.5	
Albumin	3.2	g/dL	2.7	3.9	
Globulin	3.7	g/dL	2.4	4	
Albumin: Globulin Ratio	0.9		0.7	1.5	
ALT	237	U/L	18	121	
AST	26	U/L	16	55	
ALP	2,656	U/L	5	160	
RESULT VERIFIED BY REPEAT ANALYSIS					
Bilirubin - Total	0.4	mg/dL		0.3	
Bilirubin - Unconjugated	0.3	mg/dL		0.2	
Bilirubin - Conjugated	0.1	mg/dL		0.1	
Cholesterol	657	mg/dL	131	345	
RESULT VERIFIED BY REPEAT ANALYSIS					
Hemolysis Index	2+				
<i>Index of N, 1+, 2+ exhibits no significant effect on chemistry values.</i>					
Lipemia Index	1+				
<i>Index of N, 1+, 2+ exhibits no significant effect on chemistry values.</i>					
SPECIMEN REQUIRED ULTRACENTRIFUGATION DUE TO GROSS LIPEMIA.					

📅 Plan

Recommend starting Denamarin Advanced, continuing Ursodiol and rechecking bloodwork in 1 month.

If Sparky develops anorexia, vomiting, diarrhea, lethargy or yellowing of the skin or MM, then we should recheck and repeat bloodwork sooner, then plan for internal med consult and abdominal ULS.

📋 Assessments

CBC- mild reticulocytosis, all else within normal limits

Profile- Markedly elevated ALP (ALP has been elevated on prior bloodwork but has increased compared to last bloodwork).

Hyperbilirubinemia, hypercholesterolemia. Mild hyperglobulinemia. Mild hypochloremia. Mild hyperkalemia.

📅 History

Owner reports that Galliprant caused Sparky to have a poor appetite and vomited, so she stopped the drug a few days before she came in for bloodwork and the Librela injection. Sparky is feeling better, back to eating normally, no vomiting.

📍

Health Status

Weight(lb): 79.20

💊

Medication

Prescribed By: Rx/Technician

Librela 20mg Injection (66.2-88.2 lbs) x 1

GAVE 1 ML SQ RIGHT HIP

Batch: 826096exp01JUN26 (Exp. 06-01-2026) x 1

💊

Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

GIVE 1 CHEW BY MOUTH EVERY 30 DAYS FOR THE PREVENTION OF HEARTWORM, FLEAS, TICKS & INTESTINAL PARASITES

🔍

Diagnostic Result

Requested By: Rx/Technician

Supplier: IDEXX VetLab Station- IVLS

Reference: US791-DR35406

Serology (5) Outcome:

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Heartworm Antigen	Negative				
Ehrlichia spp.	Negative				
Lyme (Borrelia burgdorferi)	Negative				
Anaplasma spp.	Negative				

🔍

Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR35407

Hematology (1) Outcome:

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
RBC	8.45	M/ μ L	5.84	8.95	
Hematocrit	56.2	%	41	60	
Hemoglobin	21.0	g/dL	14.6	21.7	
MCV	67	fL	62	76	
MCH	24.9	pg	22.1	26.7	
MCHC	37.4	g/dL	32.3	38	
RDW	17.2	%	10	19	
% Reticulocytes	1.8	%			
Reticulocytes	152	K/ μ L	21	140	
Reticulocyte Hemoglobin	25.2	pg	23.8	28.3	
WBC	11.3	K/ μ L	5.8	16.2	
% Neutrophils	73.1	%			
% Lymphocytes	20.6	%			
% Monocytes	3.3	%			
% Eosinophils	2.9	%			

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
% Basophils	0.1	%			
Neutrophils	8.26	K/ μ L	3.004	9.741	
Lymphocytes	2.328	K/ μ L	0.98	4.2	
Monocytes	0.373	K/ μ L	0.145	0.736	
Eosinophils	0.328	K/ μ L	0.141	1.927	
Basophils	0.011	K/ μ L		0.1	
Platelets	349	K/ μ L	120	412	

Remarks

No abnormalities seen upon microscopic blood film review by technician.

Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR35407

Chemistry (2) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Glucose	93	mg/dL	63	114	
IDEXX SDMA	9	μ g/dL		14	

SDMA and creatinine are within the reference interval: impairment of GFR is unlikely. Recommended next step: evaluate complete urinalysis.

Creatinine	1.2	mg/dL	0.5	1.5	
BUN	30	mg/dL	9	31	
BUN: Creatinine Ratio	25.0				
Phosphorus	4.1	mg/dL	2.5	6.1	
Calcium	11.0	mg/dL	8.4	11.8	
Sodium	145	mmol/L	142	152	
Potassium	5.3	mmol/L	4	5.4	

Na: K Ratio	27		28	37	
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Chloride	108	mmol/L	108	119	
TCO2 (Bicarbonate)	19	mmol/L	13	27	
Anion Gap	23	mmol/L	11	26	
Total Protein	7.4	g/dL	5.5	7.5	
Albumin	3.4	g/dL	2.7	3.9	
Globulin	4.0	g/dL	2.4	4	
Albumin: Globulin Ratio	0.9		0.7	1.5	
ALT	76	U/L	18	121	
AST	63	U/L	16	55	
ALP	1,541	U/L	5	160	

GGT	3	U/L		13	
Bilirubin - Total	0.3	mg/dL		0.3	
Bilirubin - Unconjugated	0.2	mg/dL		0.2	
Bilirubin - Conjugated	0.1	mg/dL		0.1	

Cholesterol	506	mg/dL	131	345	
Creatine Kinase	984	U/L	10	200	

Hemolysis Index 3+

Index of 3+ may decrease ALP and Direct Bilirubin by 25 to 50% and increase AST by 25 to 50%.

Lipemia Index N

Index of N, 1+, 2+ exhibits no significant effect on chemistry values.

Assessments

CBC- mild reticulocytosis, all other values are within normal limits

Profile- ALT has returned to normal, ALP is elevated but has decreased from the previous bloodwork, AST is mildly elevated, CK is mildly elevated. Moderate hypercholesterolemia.

Differential diagnoses for elevated ALP:

Metabolic diseases with secondary hepatopathy

- Hyperadrenocorticism (Cushing's disease)

Liver disease

- Infectious bacterial cholangiohepatitis
- Inflammatory
 - Primary (e.g. chronic hepatitis, cholangiohepatitis)
 - Secondary (e.g. pancreatitis, gastroenteritis)
- Toxic
- Vascular anomaly
- Portosystemic shunt (primary or acquired)
- Microvascular dysplasia (PSS, microvascular dysplasia)
- Neoplasia (Primary liver cancer, Metastasis to liver)
- Copper storage disease (e.g. Bedlington terriers)
- Lysosomal storage disorders

Extrahepatic cholestasis

- Pancreatitis
- Biliary disease (e.g. mucocele, gall bladder stones, cholecystitis, biliary cancer)

Monitor for signs of liver disease, gall bladder disease or pancreatitis (vomiting, diarrhea, poor appetite, weight loss) or Cushing's disease (drinking or urinating excessively). If your pet does not have any of these signs, then monitoring the ALP is reasonable at this time. If the ALP continues to elevate or if your pet becomes symptomatic then next step to investigate is abdominal ULS. Also consider ACTH stim. T4 was screened at MedVet 8/2025 and was normal. Owner reports that Cushing's test was done 5 years ago at a previous vet when liver enzymes started to elevate and was not indicative of Cushing's then.

Discussed starting a low fat diet such as Science Diet I/D lowfat or RC Lowfat GI diet., Purina EN Lowfat. Sparky is doing well currently, so opted not to change diet at this time. Owner is feeding Beneful weight control.

History

Current meds:

Ursodiol 250 mg tabs- 1 tablet by mouth every 12 hours

Librela inj for DJD

Owner reports that Sparky is doing well

Monday the 27th of April 2026

Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 60

Ursodiol 250 mg Give 1 tablet by mouth every 12 hours Refill: 12 (twelve)

Monday the 4th of May 2026

Health Status

Weight(lb): 80.20

History

Doing well at home. Update vaccines.

Check new lump on stomach.

Current medications:

Ursodiol 250 mg- Give 1 tablet by mouth every 12 hours

Simparica Trtio and Librela.

Medication

Prescribed By: Ann Everett

Simparica Trio Chewable 44.1-88lbs (per chew) x 1

Give 1 chew by mouth every 30 days for the prevention of heartworm, fleas, ticks & intestinal parasites

Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: 7/9

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Both stifles are thickened and bowed with decreased ROM.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

2 cm soft, fixed SQ mass on ventral sternum, FNA- adipocytes only, consistent with a lipoma.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal

Neurologic: No apparent abnormalities

Diagnostic Result

Requested By: Ann Everett

Supplier: In House Diagnostics

Reference: US791-DR35846

Cytology- Fine Needle Aspirate **Outcome:**

2 cm soft, fixed SQ mass on ventral sternum, FNA- adipocytes only, consistent with a lipoma.

Clinic Notes / Specifics:

Assessments

Historical elevated ALP on bloodwork, treated currently with ursodiol and clinically doing well.
DJD, managed with Librela injections

Plan

Updated DHP and Lepto vaccines
Recommend HW and flea/tick prevention.

Tuesday the 9th of June 2026

 Medication

Prescribed By: Rx/Technician

Simparica Trio Chewable 44.1-88lbs (per chew) x 2

GIVE 1 CHEW BY MOUTH EVERY 30 DAYS FOR THE PREVENTION OF HEARTWORM, FLEAS, TICKS & INTESTINAL PARASITES

Wednesday the 10th of June 2026

 Health Status

Weight(lb): 78.90

 Medication

Prescribed By: Rx/Technician

Librela 20mg Injection (66.2-88.2 lbs) x 1

Gave 1 ml SQ right shoulder (location)

Batch: 846708EXP30APR2027 (Exp. 04-30-2027) x 1

Wednesday the 22nd of July 2026

 Health Status

Weight(lb): 75.60

 Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX VetLab Station- IVLS

Reference: US791-DR37230

Urinalysis (3) Outcome:

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Collection Method	Free Catch				
Color	Straw				
Clarity	Clear				
Specific Gravity	1.010				
pH	8.0				
Urine Protein	500	mg/dL			
Glucose	neg				
Ketones	neg				
Blood / Hemoglobin	50	Ery/ μ L			
Bilirubin	neg				
Urobilinogen	norm				
Leukocyte Esterase	25	Leu/ μ L			
White Blood Cells	10 /HPF				
Red Blood Cells	2 /HPF				
Bacteria, Cocci	None detected				
Bacteria, Rods	None detected				
Squamous Epithelial Cells	<1 /HPF				
Non-Squamous Epithelial Cells	1 - 2 /HPF				
Hyaline Casts	None detected				
Non-Hyaline Casts	None detected				

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Calcium Oxalate Dihydrate Crystals	None detected				
Struvite Crystals	None detected				
Ammonium Biurate Crystals	None detected				
Bilirubin Crystals	None detected				
Unclassified Crystals	None detected				

History

PU/PD.

For the last week urinating more often.

Owner is noticing puddles of urine around the house in areas where Sparky has been laying.

Dripping urine when she is walking around the house.

No strong odors, discoloration, or blood noted per owner.

Drinking more than normal, owner having to refill water bowl often.

Eating normally, eats Meijer brand dry kibble and canned food w water added.

Small amount of diarrhea a few days ago, owner thinks it is from new treats. BMs back to normal.

No vomiting.

Normal activity levels.

Current medications:

Ursodiol 250 mg - 1 tab by mouth BID

Simparica Trio Chewable 44.1-88lbs

Librela 20 mg Injection last had on 6/10

Medication

Prescribed By: Ann Everett

Amoxicillin/Clavulanate 500mg/125mg mg Tablets (per tab) x 10

GIVE 1 TABLET(S) BY MOUTH EVERY 12 HOURS UNTIL GONE. ANTIBIOTIC- GIVE WITH FOOD.

Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: 7/9

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Both stifles are thickened and bowed with decreased ROM.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

2 cm soft, fixed SQ mass on ventral sternum, previous FNA- adipocytes only, consistent with a lipoma.

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal

Neurologic: No apparent abnormalities

Assessments

UA- no bacteria but RBC's and WBC's are present on sediment review. Urine is isosthenuric with proteinuria present.

Differential diagnoses: urinary incontinence vs UTI

Plan

Urine culture/sensitivity pending

Start Amoxi/Clav 5 day course pending C/S results

If no improvement with antibiotic and urine C/S shows no growth, then should recheck bloodwork to monitor liver enzymes and consider adding DES.

Friday the 24th of July 2026

Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR37234

Microbiology (8) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
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*Clinical History:

Leaking urine, no bacteria but RBC's and WBC's present on UA and urine is isosthenuric with proteinuria

Source: CYSTOCENTESIS

Culture Results:

Status: FINAL

No Growth

INTERPRETATION KEY for Antibiotic Susceptibility Results (when performed)

For all sources and sites, susceptibilities are reported based on MIC breakpoints where available. These standards have been established by the Clinical and Laboratory Standards Institute (CLSI). It is important to check local regulations prior to selecting an antibiotic.

S, S* = Susceptible. Organism is inhibited by the usual recommended dose.

I = Intermediate. Organism is inhibited only by the maximum recommended dose.

R, R* = Resistant. Organism is resistant to the maximum recommended dose.

SDD = Susceptible, Dose Dependent. Organism is inhibited by a specific dose. Only applies to canines. Identify the MIC value listed in the lab report and refer to the corresponding row in the table below to identify the antimicrobial dose expected to inhibit the organism

TF = To Follow. Susceptibility testing for this antibiotic is performed by Kirby-Bauer and results will follow shortly.

N/R (not reported) will appear and/or MIC data may be left blank if susceptibility testing is not feasible or clinically appropriate.

Enrofloxacin Dosing Guidelines (Dog Only):

*If the MIC is $\leq 0.06 \mu\text{g/mL}$, the interpretation is Susceptible (S) and the recommended dose is 5 mg/kg orally every 24 hours, or 2.5 mg/kg orally every 12 hours.

*If the MIC is $0.12 \mu\text{g/mL}$, the interpretation is Susceptible Dose-Dependent (SDD) and the recommended dose is 10 mg/kg orally every 24 hours.

*If the MIC is $0.25 \mu\text{g/mL}$, the interpretation is Susceptible Dose-Dependent (SDD) and the recommended dose is 20 mg/kg orally every 24 hours.

Marbofloxacin Dosing Guidelines (Dog Only):

*If the MIC is $\leq 0.12 \mu\text{g/mL}$, the interpretation is Susceptible (S) and the Recommended dose is 2.75 mg/kg orally every 24 hours.

*If the MIC is $0.25 \mu\text{g/mL}$, the interpretation is Susceptible Dose-Dependent (SDD) and the recommended dose is 5.5 mg/kg orally every 24 hours.

*The SDD category only appears for canine results with growth of Enterobacterales, Pseudomonas aeruginosa, and Staphylococcus spp. For more information, please see the "Microbiology Guide to Interpreting Antimicrobial Susceptibility Testing (AST)" at www.idexx.com/MIC.

Saturday the 25th of July 2026

Assessments

Urine culture/sens- no growth.
Urine leaking is likely incontinence, likely exacerbated by isosthenuric urine.

Plan

Recommend starting DES 1mg caps- 1 cap by mouth SID x5d, then 1 by mouth once weekly if incontinence is still occurring. Call prescription in to Stokes/Hill's compounding pharmacy.

Recommend discontinuing Librela injections for now, in case the incontinence is a side of effect of Librela.

Recommend bloodwork, Chem Panel 4, IDEXX CODE 2240 (want to have ciALP in bloodwork) to investigate isosthenuric urine.

Friday the 28th of August 2026

Health Status

Weight(lb): 77.80

Saturday the 29th of August 2026

Diagnostic Result

Requested By: Ann Everett

Supplier: IDEXX Reference US

Reference: US791-DR37852

Chemistry (2) **Outcome:**

Clinic Notes / Specifics:

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Glucose	99	mg/dL	63	114	
IDEXX SDMA	10	µg/dL		14	
SDMA and creatinine are within the reference interval: impairment of GFR is unlikely. Recommended next step: evaluate complete urinalysis.					
Creatinine	1.4	mg/dL	0.5	1.5	
BUN	37	mg/dL	9	31	
Phosphorus	4.3	mg/dL	2.5	6.1	
Calcium	10.8	mg/dL	8.4	11.8	
Sodium	146	mmol/L	142	152	
Potassium	5.1	mmol/L	4	5.4	
Chloride	109	mmol/L	108	119	
TCO2 (Bicarbonate)	16	mmol/L	13	27	
Total Protein	7.1	g/dL	5.5	7.5	
Albumin	3.3	g/dL	2.7	3.9	
Globulin	3.8	g/dL	2.4	4	
Albumin: Globulin Ratio	0.9		0.7	1.5	
ALT	44	U/L	18	121	
AST	21	U/L	16	55	
Corticosteroid-Induced ALP	730	U/L		35	
ALP	1,028	U/L	5	160	
GGT	< 1	U/L		13	<
LDH	99	U/L	50	380	
Bilirubin - Total	0.3	mg/dL		0.3	
Cholesterol	427	mg/dL	131	345	
Triglyceride	280	mg/dL	20	150	
TRIGLYCERIDE VALUE OBTAINED BEFORE ULTRACENTRIFUGATION.					
Amylase	1,029	U/L	337	1469	
Lipase	201	U/L		250	
Creatine Kinase	73	U/L	10	200	
Hemolysis Index	2+				

Test	Results	Unit	Lowest Value	Highest Value	Qualifier
Index of N, 1+, 2+ exhibits no significant effect on chemistry values.					
Lipemia Index	N				
Index of N, 1+, 2+ exhibits no significant effect on chemistry values.					
SPECIMEN REQUIRED ULTRACENTRIFUGATION DUE TO GROSS LIPEMIA.					

Tuesday the 1st of September 2026

Assessments

Profile- ALP is markedly elevated, ciALP is markedly elevated, Moderate hypercholesterolemia, hypertriglyceridemia

Differential diagnoses for elevated ALP:

Metabolic diseases with secondary hepatopathy

- Hyperadrenocorticism (Cushing's disease)

Liver disease

- Infectious bacterial cholangiohepatitis
- Inflammatory
 - Primary (e.g. chronic hepatitis, cholangiohepatitis)
 - Secondary (e.g. pancreatitis, gastroenteritis)
- Toxic
- Vascular anomaly
- Portosystemic shunt (primary or acquired)
- Microvascular dysplasia (PSS, microvascular dysplasia)
- Neoplasia (Primary liver cancer, Metastasis to liver)
- Copper storage disease (e.g. Bedlington terriers)
- Lysosomal storage disorders

Extrahepatic cholestasis

- Pancreatitis
- Biliary disease (e.g. mucocele, gall bladder stones, cholecystitis, biliary cancer)

Sparky has symptoms of Cushing's disease (PU/PD). Recommend ACTH stimulation or and referral for internal medicine consult for abdominal ULS. T4 was screened at MedVet 8/2025 and was normal. Owner reports that Cushing's test was done 5 years ago at a previous vet when liver enzymes started to elevate and was not indicative of Cushing's then, but Cushing's disease testing can change with time, so retesting is warranted.

Recommend starting a low fat diet such as Science Diet I/D lowfat or RC Lowfat GI diet., Purina EN Lowfat. Sparky is currently eating Beneful weight control.

Medication

Prescribed By: Ann Everett

EXTERNAL DISPENSE PRESCRIPTION x 1

HILL'S PRESCRIPTION DIET I/D LOWFAT CANINE FOOD- CANNED OR DRY ROYAL CANIN GASTROINTESTINAL LOWFAT CANINE FOOD- CANNED OR DRY, PURINA VETERINARY DIET EN LOWFAT CANINE FOOD- CANNED OR DRY, FEED AS DIRECTED REFILL AS NEEDED UNTIL 9/1/2027

Monday the 21st of September 2026

Health Status

Weight(lb): 77.50

History

owner noticed Sparky's ear was swollen on Sunday, wasn't shaking her head before that.
O also noticed 2 new lumps: front right and under her chest.
Feeding Hill's Low Fat wet and dry.

Current medications: Ursodiol 250mg- 1 BID, owner gave am dose.

Physical Exam

General Appearance: Bright, alert and responsive.

Body Condition Score: 7/9

Hydration: Hydration appears normal.

Eyes: Corneas clear, pupils normal size, symmetrical, sclera white, no ocular discharge.

Ears: No exudate observed, no redness present in canals.

Aural hematoma involving rostral, distal 1/3 of ear pinna AS.

Oral Cavity: Teeth are free from excessive tartar, no gingivitis present.

Nasal Cavity: No obvious abnormalities observed.

Cardiovascular: Regular rhythm; no murmur detected.

Respiratory: Eupneic, normal bronchovesicular sounds in all fields.

Abdomen: Abdomen palpates normally.

Rectal: Did not perform rectal exam.

Musculoskeletal: Both stifles are thickened and bowed with decreased ROM.

Integument: Normal amount of shedding; skin looks normal; hair coat in good condition.

2 cm soft, fixed SQ mass on ventral sternum, previous FNA- adipocytes only, consistent with a lipoma. Multiple other soft SQ masses, lipoma-like, that the owner is monitoring

1.5 cm cutaneous mass on right dorsal antebrachium, FNA- mast cell tumor

Lymph Nodes: Lymph nodes are all normal in size.

Urogenital: External genitalia appears normal

Neurologic: No apparent abnormalities

Therapeutic / Procedure

Aural Hematoma - Drain during exam

Specifics:

Drained aural hematoma left ear

Diagnostic Result

Requested By: Ann Everett

Supplier: In House Diagnostics

Reference: US791-DR38196

Cytology- Fine Needle Aspirate **Outcome:**

1.5 cm cutaneous mass on right dorsal antebrachium, FNA- mast cell tumor

Clinic Notes / Specifics:

Assessments

Elevated ALP on bloodwork, is on ursodiol and has internal med consult this Wednesday (in 2 days)

DJD, managed with Librela injections

Aural hematoma AS
Mast cell tumor right front leg.

Plan

Drained aural hematoma. Recommend owner monitor for it to fill again and if it does we can drain or place a teet canula.

Recommend surgical excision with biopsy of MCT

Proceed with internal med consult this week as planned.



Agri-Pet Veterinary Service
1447 Millville Shandon Road
Hamilton, Ohio, 45013
1447 Millville Shandon Road

10-03-2026

Client: Lindsay Klug
Address: 1515 Brookcrest DR
Hamilton, Ohio, 45013
Patient: Sparky
Species: Canine (Dog)
Breed: Pitbull

Patient ID: 143977
Phone Number: Lindsay: 513-807-4414
Age: 9 years 2 days
Sex: FS
Color: Black

Vaccination and Treatment History

This is a record of vaccination for Sparky Klug

a Canine (Dog) Pitbull of age 9y 0m

Sparky has received the following vaccinations and standard treatment:

Veterinary Service(s)	Fulfilled With	Fulfilled At	Due on
Fecal			11-25-2017
Heartworm Prevention	Simparica Trio Chewable 44.1-88lbs (per chew)	06-09-2026	08-04-2026
Heartworm Test	IVLS- Heartworm and Tick Disease Test	04-09-2026	04-09-2027
Leptospirosis Vaccine	Leptospirosis Vaccine Annual	05-04-2026	05-04-2027
Dental Check	Exam - Annual Physical (Well Pet)	05-04-2026	05-04-2027
Wellness Exam	Exam - Annual Physical (Well Pet)	05-04-2026	05-04-2027
Rabies Vaccine	Rabies Vaccine 3 year	04-08-2025	04-08-2028
DHPP Vaccine	DH- Parvo 3 Year	05-04-2026	05-03-2029
Microchip			04-01-2018
Bordetella Vaccine	Bordetella Annual - Bronchi-Shield - Oral	09-20-2024	09-20-2025
Wellness Blood and Urine Screen	Adult Wellness Select	02-23-2026	08-24-2026
Canine Influenza Vaccine			To Do
Lyme Vaccine			To Do

If applicable, Sparky is due for booster vaccinations or flea/tick/Heartworm treatment of the following **in the next 30 days:**

Date of Certificate: 10-03-2026